جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمة محمد جودة حبيب عبدالحميد
المحافظة : القاهره **الحي / المدينة :** المرج **العنوان :** //
رقم البطاقة : 28807010102126 **رقم التليفون :** 01009941373 **تاريخ تسجيل الإستمارة :** 21-10-2026
العمر(تاريخ الميلاد) : 01-07-1988 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمة محمد جودة حبيب عبدالحميد
National ID: 28807010102126

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

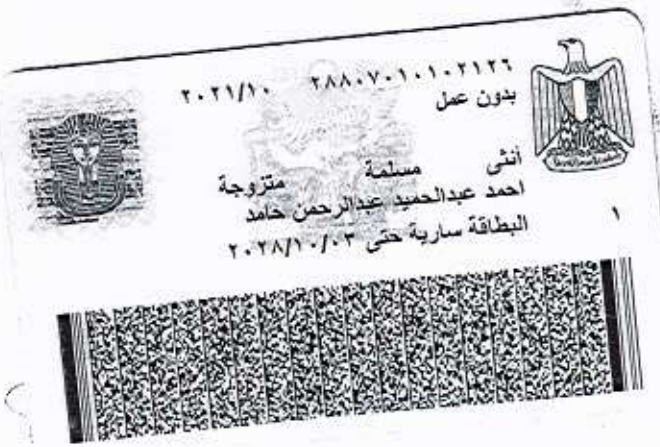
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
فاطمه
محمد جوده حبيب عبدالحميد مشالى
٣ش محمد مامون-محمد نجيب
المرج - القاهره
٢٨٨٠٧٠١٠١٢١٢٦
JB5009437


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بدون عمل
انثى
مسلمة
متزوجة
احمد عبدالحميد عبدالرحمن حامد
البطاقة سارية حتى ٢٠٢٨/١٠/٠٣

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

04/07/2026

MDT FORM

Name:	Fatma Mohamed Gouda .	**MRN:**	1089
Age:	38 Y	**ID:**	28507010102126
Menopausal Status:	pre	**Phone:**	01009941373
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	free	**Reason Of MDT:**	DP

Clinical Data:

BX: IDC GII

ER 8/8 **PR** +ve **HER** 0 **KI67**

Sonomammography:

Mets Workup:

History:

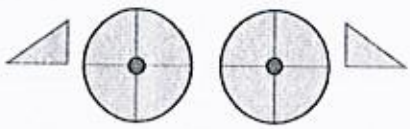
(2021)Ct3N1 >> received NACT AC *4 TXT * 4 >> BCS >> yct1 n0 >>PORT >> tam +zoladex till 8-2025 >> then developed HFLS >> BX gata +ve , Er 8/8 pr 2/8 her2 +2 amplified >> taxol *17 wks ,

Recommendations:

MDT Outcome & Plan

For Tykerb - Xeloda
Consider TDM-1 in NGO

R L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد النيرلوي	د/ ابو بكر حجازي			

Name: فاطمه محمد جوده	
MR. : 1089	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presents with metastatic breast cancer to the liver and bone (sternal). Initial pathology from a hepatic lesion (HFL) biopsy on 8/8/2025 showed HER2 2+ by SISH, with a large HFL and multiple satellite lesions. The patient had prior right-sided breast cancer treated with WLE in 2021, adjuvant tamoxifen and zoledronic acid until 9/2025, and now has liver and bone metastases. Beginning of systemic therapy was initiated with Taxol 140 mg on cycle 3 (C3) and Herceptin 440 mg on the same cycle, with zoledronic acid 15 mg given on 15/1 and an ejection fraction of 65% noted on 2/12.

The next scheduled dose was Herceptin 440 mg on cycle 4 (C4) and Taxol 140 mg on week 11 (W11), with a CT and NRI brain scan requested. Pharmacy records confirm administration of Taxol 140 mg and Herceptin 440 mg during week 11, cycle 4.

On 23/02/2026, the patient's disease was described as metastatic breast cancer to bone and liver, with a liver biopsy confirming breast origin. The plan was to administer Taxol 150 mg on week 12 (W12) and week 13 (W13), with labs and Herceptin on cycle 5 (C5) and an echocardiogram on week 14 (W14). A CT on 16/2 was requested but no comparison was available. Imaging noted a sternal lytic lesion and a 6.4 × 1.7 cm hypodense lesion in hepatic segment II.

Day-care pharmacy visits on 02/03/2026 and 09/03/2026 documented administration of Taxol 150 mg on weeks 13 and 14, respectively, with Herceptin 440 mg on cycle 5 and a repeat echocardiogram and laboratory work on day 2/3. Labs were accepted.

On 23/03/2026, the patient received Taxol 150 mg on week 16 (W16) and Herceptin 440 mg on cycle 6 (C6). Subsequent pharmacy visits on weeks 17 and 18 were scheduled.

A CT of the chest, abdomen, and pelvis on 30/03/2026 showed no new disease progression. The patient reported vaginal bleeding and urinary incontinence; a vaginal ultrasound and urine analysis were performed, and a brain MRI on 14/3/2026 was normal.

On 13/04/2026, a multidisciplinary team meeting on 7/4/2026 discussed initiation of Tykerb (erlotinib) plus Xeloda (capecitabine) pending Ministry of Health approval. The first cycle of Xeloda and Tykerb was started on 21/04/2026 with a dosing schedule of Xeloda 4 + 3 × 14 days and Tykerb 5 × 1 × 21 days. Tykerb was administered on 30/04/2026, and a trans-vaginal ultrasound on 04/2026 noted multiple lower uterine varicosities but otherwise normal findings.

On 16/05/2026, the patient received Xeloda 4 + 3 × 14 days and Tykerb 5 × 1 × 21 days, with pharmacy dispensing on 30/5/2026. On 27/06/2026, the patient was scheduled to continue Tykerb and Xeloda after laboratory review, with a follow-up planned after three weeks.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

On 03/08/2026, the patient was to continue Tykerb/Xeloda after laboratory results and new CTs and bone scan were requested. On 02/09/2026, laboratory results for Tykerb 5 tablet regimen were accepted, and a CT of the chest, abdomen, and pelvis along with a bone scan were pending.

The patient has completed 6 cycles of Taxol and 6 cycles of Herceptin, and has reached week 18 of therapy.

GUSTAVE ROUSSY
INTERNATIONAL

Name : Fatema Mohamed Goda
Branch : Portable
Ref.Physician Prof. : cap+c

Date : 16-02-2026 03:21
Medical No. : GRI-170900
Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST: compared tom last study dated 9/9/2025 current study reveals

- Still seen right centimetric axillary lymph nodes with effaced fatty hilum largest 17mm
- Newly developed Sternal mixed lytic/sclerotic lesions with mottled osseous texture
- Clear both lungs showing no sizable pulmonary nodules, consolidation or cavitation.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- No gross cardiomegaly

II- ABDOMEN AND PELVIS:

- Average sized liver showing progressive course of the previously noted multiple hypodense hepatic focal lesions to be seen at both lobes , largest one seen at segment II measuring about 6.4x1.7. small sattellites seen at right lobe segment V, compared to left lobe affection only in last study with largest lesion 22 x21 mm
- No or dilated biliary radicles.
- Cholecystectomy clip.
- Normal C.T appearance of the spleen, pancreas, both kidneys, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No pathologically enlarged abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ***Generalized progressive course as regard the CT finding of the chest and pelvi-abdominal CT study.***

Dr Abubakr Hejazi MSc FEBR

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name : Fatema Mohamed Goda	**Date : 16-02-2026 03:21**
Branch : Portable	**Medical No. : GRI-170900**
Ref.Physician Prof : cap+c	**Ins.Company :**

- Right subcentimetric axillary lymph nodes with effaced fatty hilum for U/S correlation
- Sternal mixed lytic/sclerotic lesions with mottled osseous texture for bone scan correlation
- multiple hypodense hepatic focal lesions for better assessment by triphasic CT

Dr. Nourhan Mohamed,MD

17/02/2026 08:00 AM

Dr. Nourhan Mohamed,MD

17/02/2026 08:00 AM

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name : Fatema Mohamed Goda	Date : 16-02-2026 03:21
Branch : Portable	Medical No. : GRI-170900
Ref.Physician Prof : cap+c	Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Right subcentimetric axillary lymph nodes with effaced fatty hilum.
- Sternal mixed lytic/sclerotic lesions with mottled osseous texture
- Clear both lungs showing no sizable pulmonary nodules, consolidation or cavitation.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- No gross cardiomegaly

II- ABDOMEN AND PELVIS:

- Average sized liver showing multiple hypodense hepatic focal lesions, largest one seen at segment II measuring about 6.4x1.7. no or dilated biliary radicles.
- Cholecystectomy clip.
- Normal C.T appearance of the spleen, pancreas, both kidneys, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No pathologically enlarged abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name : Fatema Mohamed Goda	**Date : 16-02-2026 03:21**
Branch : Portable	**Medical No. : GRI-170900**
Ref.Physician Prof : cap+c	**Ins.Company :**

- Right subcentimetric axillary lymph nodes with effaced fatty hilum for U/S correlation
- Sternal mixed lytic/sclerotic lesions with mottled osseous texture for bone scan correlation
- multiple hypodense hepatic focal lesions for better assessment by triphasic CT

Dr. Nourhan Mohamed,MD

17/02/2026 08:00 AM

Dr. Nourhan Mohamed,MD

17/02/2026 08:00 AM

ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / فاطمة محمد جودة	Request No.	: 13006774
Patient No.	: 28807010102126	Request Date	: 16/10/2025
Path No.	: 3167/25	Verification Date	: 26/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left heptic lobe mass lesion, in a case with history of breast cancer.
Nature of Specimen: Left hepatic lobe focal lesion, Tru-cut biopsy, collected on 15/10/2025.

Gross Examination:
- Two grey white tissue cores, each was 1 cm long. Totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5), GATA3 (L50-823) & SISH.

Microscopic Examination:
* Histopathological examination reveals cores of tumor formed of malignant epithelial cells showing moderate nuclear pleomorphism and eosinophilic cytoplasm, arranged in clusters with moderate tendency to tubular formation among desmoplastic stroma.

* IPT (Positive control showed appropriate reactivity):
- GATA3: Diffuse strong positive staining in tumor cells.
- ER: Strong nuclear staining in 90% of tumor cells (score 8/8).
- PR: Weak nuclear staining in less than 1% of tumor cells (score 2/8).
- Her2: Moderate incomplete membrane staining in more than 10% of tumor cells (score 2+).
- SISH: Amplified.
(Average Her2 copy No.: (4.4), Her2/CEP17 ratio: (3.03)).

Diagnosis:
Left hepatic lobe focal lesion, Tru-cut biopsy:
Findings are consistent with metastatic carcinoma of breast origin.
IPT:
- GATA3: Strong positive staining in tumor cells.
- ER: Positive (score 8/8).
- PR: Negative (score 2/8).

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
INTERNATIONAL CANCER

Name : fatma mohamed goda
Branch : Portable

Ref.Physician Prof : brain+c

Date : 14-03-2026 03:50
Medical No. : GRI-260314155022STD131221107523748902
Ins.Company :

OPEN MR EXAMINATION OF THE BRAIN

TECHNIQUE:



Axial T1, T2, FLAIR, diffusion & ADC WIs
Coronal T2WIs
Sagittal T1WIs
Post contrast T1 WIs

FINDINGS:

- Normal size and configuration of the ventricular system.
- No midline shift or deformity.
- No evidence of cerebral, cerebellar or brain stem areas of abnormal signal.
- Normal cortical sulci and extra axial CSF spaces.
- No evidence of extra axial collections.
- No cerebello pontine angle masses.
- No evidence of posterior fossa abnormalities.
- Intact cervico medullary junction.

OPINION:

Normal MRI of the brain.

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name: Fatma Mohamed Goda Habib Abdel Hameed Mashali	Date : 09/09/2025
Exam : triphasic C.T Liver	Age : 37 Year(s)
Medical ID: 2842/2021	Pat. ID: 17090

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

**** A reported case of breast cancer,**

POST CONTRAST CT OF THE Chest:

- **Multiple right axillary enlarged L. Ns the largest seen at level I measures 18 x12mm**
- No sizable suspicious pulmonary nodules or masses.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Still noted right thyroid lobe large nodule with retrosternal extension

- *Triphasic C.T. scan of the abdomen & pelvis*

- ***Average sized liver showing homogeneous hypodensity. Large enhancing focal infiltrative lesion is seen involving most of the left lobe with areas of breakdown . multiple variable sized satellites are noted with the largest seen at segment IV b measuring 22x 20 mm.**
- ***Cholecystectomy clips are seen at the oprative bed.**
- ***No dilated intra or extra-hepatic bile ducts .**
- ***Patent main portal vein trunk and both branches.**
- ***Average sized spleen , no focal lesion with small splenule**
- ***Normal C.T. appearance of the pancreas , adrenal glands, both kidneys ,bowel loops & other pelvic organs.**
- ***Urinary bladder shows no masses or stones.**
- ***No retroperitoneal lymphadenopathy .**
- ***No ascites .**
- **Left iliac sclerotic lesion**

Opinion:

- **Right breast cancer with axillary lymphadenopathy .**
- **Sternal destructive lesion with left iliac sclerotic lesions likely metastatic .**
- **Enlarged fatty liver with multiple hepatic focal lesions likely metastatic**

Dr/Reham Khedr,,,MD

Signature Reeham.Khader
15/09/2025 11:25 PM

فورمات

Name : Fatima Mohammed Joudeh Habib Abdulhamid	**Date : 17-09-2026 01:26**
Branch : Dokki	**Medical No. : 463959**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : تعاقد مستشفيات**

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 38-year-old female with right breast carcinoma, status post multimodal therapy. Referred for follow-up evaluation of sternal and pelvic findings on recent CT (August 26, 2026) compared to prior bone scan dated September 13, 2025.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Intense solitary linear radiotracer uptake is localized within the sternal body, corresponding to the sclerotic lesion reported on CT and persistent uptake on the prior skeletal scintigraphy.
- No focal hyperactive or photopenic lesions are identified in the iliac bones or pelvic ring corresponding to the tiny lytic foci reported on CT.
- The remainder of the axial and appendicular skeleton shows uniform age-appropriate tracer distribution without additional focal osseous lesions.
- Mild non-specific osteoarthritic changes are noted in the lumbar spine and weight-bearing joints.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Solitary high-uptake sternal lesion, persistent on prior study and corresponding to the CT sclerotic change—differential includes focal metastasis versus benign sclerotic process (e.g., prior trauma/post-radiation response), for further assessment.
- No osteoblastic correlate for the left iliac lytic foci on CT.
- No scintigraphic evidence of widespread skeletal metastases.

Dr. Alaa Nagi

MD Nuclear Medicine

NILE DIAGNOSTIC CENTER

Call Center 19656

Bone Scan
[[DOCTYPE:BONE_SCAN]]